Digital Mental Health and Psychological Support System for Students in Higher Eduation

Design Thinking for innovation & Entrepreneurship (DTIE)

Section : 5

Project Batch- 02

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Project: MindBridge - Digital Mental Health and Psychologicl Support System for Student in Higher Education

Vision and Mission

Vision

Every student in higher education has private, stigma-free, 24x7 access to mental-health support - from the first signs of stress to professional care.

MindBridge: a bridge from "I'm not okay" to help the right help.

Mission

→ Make early self-checks and daily support effortless and anonymous.
→ Connect students to campus counsellors and national helplines in a few taps.
→ Give institutions anonymised insights insights so they can act before a crisis.
→ Keep privacy, consent and safety at the centre of every feature.

Problem Statement

Statement: Students facing academic pressure, isolation and financial stress have no private, always-available way to ask for help early. Campus counselling is scarce and visible to peers, so distress goes unnoticed until it becomes a crisis.

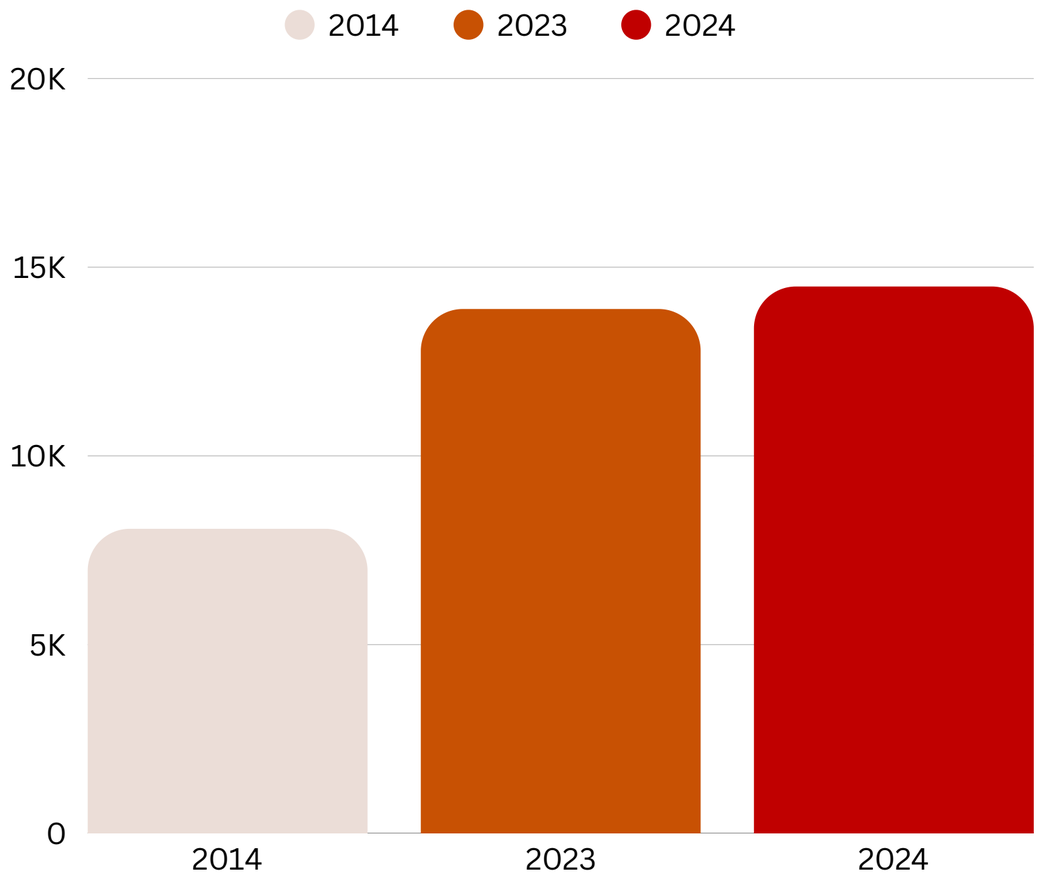
14,488 student suicides in India in 2024, about 40 a day (NCRB, ADSI 2024).

65% of surveyed institutions had no access to mental health professionals (Source: National Task Force interim report, cited in the Supreme Court order of January 2026).

About 3 in 4 institutions had no full-time counsellor (Source: National Task Force interim report, cited in the Supreme Court order of January 2026).

Root causes: stigma, too few counsellors, late detection.

Research / Facts / Market & Industry Study



Facts:

- 1 in 7 adolescents aged 10–19 lives with a mental disorder (WHO; verify this figure before presenting).
- Most mental health conditions emerge before age 24 (National Task Force, via the Supreme Court judgment).
- Tele-MANAS had handled 43.25 lakh+ calls by August 2026, so demand for remote support is proven. Manodarpan had 3.32 lakh+ calls by March 2026.
- UGC regulations (13 Jan 2026) and Supreme Court directions (15 Jan 2026) now hold institutions accountable. They require annual reporting on student suicides and round-the-clock medical access on residential campuses.
- India has the world's second-largest higher-education system by enrolment, so the market is large and newly regulated.

Target User - Customer

Students (primary user):

- UG/PG students aged 17–25, especially first-years, hostellers and exam-stressed learners.
- They need private, judgement-free support.
- They need help at 2 a.m., not only in office hours.
- They need to know when stress calls for a professional.

Counsellors and mentors (secondary user):

- They need triage so urgent cases come first.
- They need scheduling and session notes in one place.
- They need early signals without breaching privacy.

Institutions (customer/payer):

- universities, colleges and deans of student welfare.
- They need to meet UGC and Supreme Court expectations.
- They need anonymised, aggregate trends.
- They need an affordable support layer that is quick to deploy.

Competitors / Benchmarks

Option	What it offers	Gap MindBridge fills
Tele-MANAS / Manodarpan	Free national helplines and psychosocial support	Not linked to the student's campus, counsellor or daily check-ins
YourDOST, Wysa	Chat/AI support and counsellor access	Individual-focused, often paid, little institution workflow
Headspace, Calm	Meditation and sleep content	Self-help only, no screening or escalation
Campus counselling cell	Trusted in-person care	Few counsellors, fixed hours, stigma
MindBridge	Check-ins, screening, booking and SOS in one flow	Links student, counsellor and institution insights

Risks and Gap

Gaps:

- Support is reactive and starts after a crisis.
- Stigma keeps students away from visible counselling.
- Most institutions lack full-time counsellors.
- Helplines, apps and campus cells don’t connect.

Risks and mitigations:

- Data privacy: anonymous IDs, encryption, consent, aligned to the DPDP Act 2023.
- Crisis missed or mishandled: the app never diagnoses, and high-risk results route to a human and to Tele-MANAS (14416).
- Low adoption: anonymous mode, orientation roll-out, peer champions.
- Over-reliance on the app: position it as a bridge to professionals, not a replacement.
- Counsellor capacity: triage queue and tele-counselling partners.

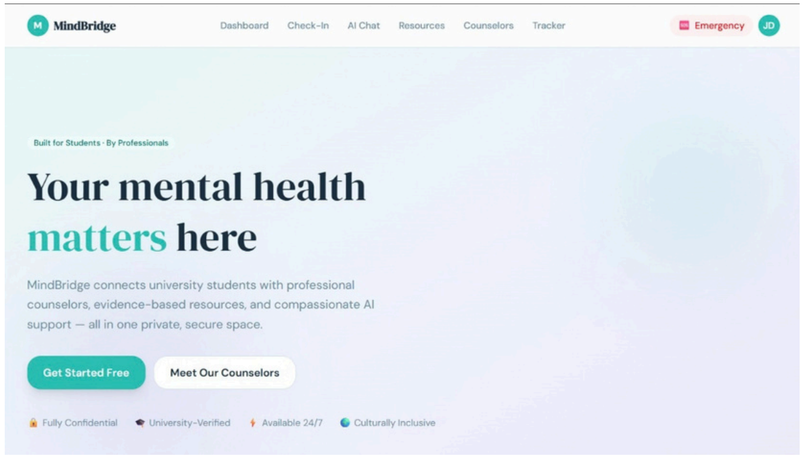
Concept/Idea/Solution

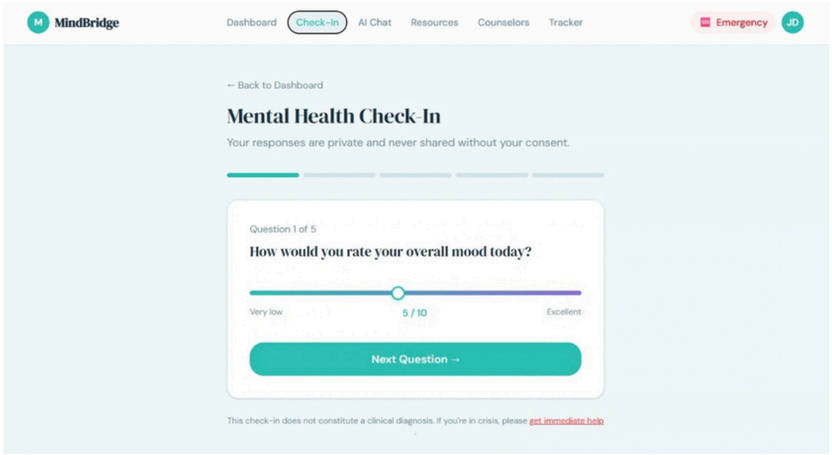
MindBridge is a private, always-on bridge from student to counsellor to crisis help.

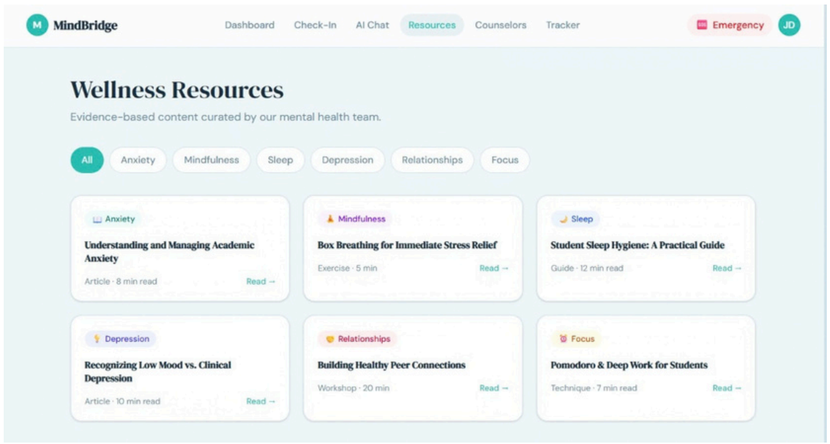
Features: these are my guesses at your scope, so edit them to match your Figma screens.

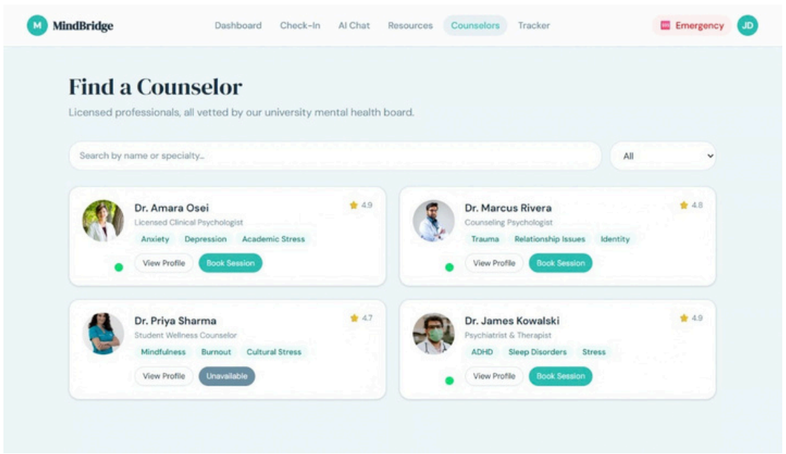
1. Mood check-in and journal: 30-second daily check-ins tracking mood, sleep and stress.
2. Confidential self-screening: standard scales (PHQ-9 / GAD-7 style) with plain-language results, never a diagnosis.
3. Counsellor connect: anonymous in-person or tele-booking, with urgent cases triaged first.
4. Peer support circles: moderated communities on exams, homesickness and relationships.
5. One-tap SOS: Tele-MANAS 14416 and the campus emergency contact.
6. Wellness toolkit: breathing, sleep and study-stress exercises.

Images/Storyboard/Architecture









Architecture (four layers):

1. Student app (web or mobile).
2. Core services: anonymous IDs, triage engine, scheduling, moderation.
3. Secure data layer: encrypted storage, consent logs, anonymised analytics.
4. Counsellor console and institution dashboard

Lean Canvas Model

Problem: stigma, too few counsellors, late detection.

Solution: anonymous check-ins, self-screening, counsellor booking, one-tap SOS.

Unfair advantage: co-designed by students, campus counsellor tie-ups, compliance-ready.

Channels: orientation and hostels, peer champions, college ERP/LMS link.

Key metrics: monthly active students, time to first support, escalations connected.

Unique value proposition: one private bridge from “I’m not okay” to the right help, built for campus life.

Revenue streams: institution licence per student per year, premium counsellor packages, grants and CSR. Students stay free.

Customer segments: students (users), counsellors and mentors, institutions (payers).

Cost structure: development and hosting, counsellor honoraria, security audit and compliance, awareness campaigns.

Capital (Financial)

- Estimated Budget: ₹30,000–₹40,000
- Team Size: 4–5 members
- Development Duration: 6–8 weeks
- Pilot Audience: 30–50 students
- Implementation Scope: One college

Major Expenses: Website development, domain and hosting, technical tools, testing, awareness materials, and contingency expenses.

Note: Budget figures are preliminary estimates for the pilot implementation.

Roadmap/Timeline

Timeline	Key Activities
Week 1	Research and requirement analysis
Week 2	UI/UX design and prototyping
Weeks 3–4	Website development
Week 5	Testing and improvements
Week 6	Pilot launch in one college
Weeks 7–8	Feedback, evaluation, and refinement

Expected Outcome: A functional MindBridge website prototype, piloted with 30–50 students, with feedback collected for future improvements.